



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمه انور محمود خليفه

العنوان : =

الحي / المدينة : اخميم

المحافظة : سوهاج

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 26005220101061 رقم التليفون : 01064315868

العمر (تاريخ الميلاد) : 22-05-1960 المهنة : بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان

الوظيفة: -----
الوظيفة: -----
الوظيفة: -----

التوقيع: -----
التوقيع: -----
التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----

قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمه انور محمود خليفه National ID: 26005220101061	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحليل النحبة

فاطمة

أنور محمود خليفه

١ حارة صابر زيان ش مسجد الصالحين
الوراق - الجيزة



٢٦٠٠٥٢٢٠١٠١٠٦١

KA5101155

Anwar A. Elshenawy
Ass. Professor Surgical Oncology
Aswan University

د/ أنور الشناوي
أستاذ جراحة الأورام م.
كلية الطب - جامعة أسوان

الإسم: فاطمة أنور محمود
التاريخ: ٢٠١٧/٤/٢١
٧٧ سنة

R/
sp panhystrectomy, omentectomy
& Excision of pelvic
peritoneal nodules.

All were positive for
Carcinoma
For adjuvant therapy

٢٠٢٣/٠٢ ٢٦٠٠٥٢٢٠١٠١٠٦١

ربة منزل

مسلمة

أنثى



البطاقة سارية حتى ٢٠٢٣/٠٦/٠٣

GUSTAVE/ROUSSY
INTERNATIONAL
EGYPT

Date Of Submission:

Chp 4
v107

Date Of MDT:

07/26/2026 15:37:47

General GRI-E MDT form

Patient Name :	فاطمة اتور محمود خليله	Age :	66 Y
Sex :	☐ Male ☒ Female	Comorbidity :	26005220101061
ID No. :	12249	Telephone :	01064315868
Family History :		Marital Status :	☒ Married ☐ Divorced ☐ Single
Menopausal Status :		Reason of MDT :	
		MDT name::	

Medical Report

Diagnosis :	malignant mixed Mullerian carcinosarcoma stage 3 of endometrium
Notes :	
History:	66 yo female patient dm , htn with postive FH (BROTHER PERITONEUM) 5 months ago vaginal bleeding mri pelvis 3/2026 -- endometrial mass lesion patient then underwent panhysterectomy + omentectomy pathology : malignant mixed Mullerian carcinosarcoma stage 3 more than half of the myometrium infiltrating upper surface + tumor deposits on right fallubian tube + bilateral ovaries
Investigations:	
Recommendations:	fir ct cap + new labs meanwhile TO START TAXOL CARBO then PORTH
MDT DECISION:	for ct cap + new labs Portcath insertion TO START TAXOL CARBO x 6 cycles then PORTH

Signature

Hematology

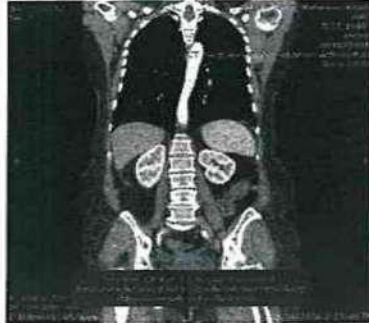
Pediatric

Bone marrow transplant

د. خالد عبد العزيز

د. محمد التبراري

Patient's Name: Fatima Anwar Mahmoud Khalifa
Date: 2026-07-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATIUN OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Clear operative bed in the pelvis with no evidences of either residual , recurrent and /or newly-developed neoplastic lesions seen herein.
- Hyperinflated both lungs.
- Patchy areas of ground-glass opacities are seen in the middle lobe and in the anterior segment of left lower pulmonary lobe.
- No sizable abnormally-enhancing pulmonary nodules ,other patchy areas of consolidations or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- A tiny focus of osteosclerosis is seen in the postero-inferior aspect of DV4 with mild spondylolisthesis of L4 over L5 and mild retrolisthesis of L5 over S1 vertebral bodies with gas hypodensity seen in L5-S1 disc space,denoting its degeneration and spondylo-degenerative changes scattering in the scanned spine.
- Average sized liver,measuring about 15.3 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 11.4 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

Patient's Name: Fatima Anwar Mahmoud Khalifa
Date: 2026-07-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Clear operative bed in the pelvis with no evidences of either residual , recurrent and /or newly-developed neoplastic lesions seen herein.
- Hyperinflated both lungs with patchy areas of ground-glass opacities in the middle lobe and in the anterior segment of left lower pulmonary lobe.likely to be inflammatory reactions.
- A tiny focus of osteosclerosis in the postero-inferior aspect of DV4,likely to be an osseous deposit in evolution.

Prof/Hanan El-Ahwal.

Dr.
Soliman Saba
M.Sc., M.D. Pathology
Prof Of Pathology
Faculty of Medicine, Cairo University
Fellow, Ohio State University U.S.A.

دكتور
سليمان سابا
أستاذ الباثولوجيا
كلية طب قصر العيني
زميل جامعة أوهايو بالولايات المتحدة الأمريكية

Name: فاطمة أنور محمود

Age: 66 years

Clinical Diagnosis: Endometrial carcinoma.

Nature of specimen: 1) Panhysterectomy with omentectomy.
2) Excision of pelvic peritoneal nodules.

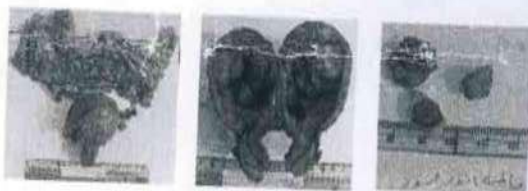
Referred By: Prof. Dr. ANWAR EL-SHINAWY.
MABARRET EL-MAADI

Date: 22/06/2026

PATHOLOGY REPORT

GROSS: Specimen received consists of:

1. A **uterus, tubes and ovaries**. The **uterus** measured 12x8x5.5 cm showing friable grayish white polypoid tumour mass; in the uterine cavity arising from the fundus and measuring 5.5x5 cm with focal areas of hemorrhage and necrosis. Tumour tissue was seen infiltrating the underlying myometrium extending beyond its half thickness without reaching to the perimetrial surface. Tumour tissue was also seen extending to the endometrial lining of lower uterine segment. The **cervix** showed fleshy whitish cut section entangling small cysts containing mucus and showing infiltration by tumour tissue. The **right and left tubes** measured 7.5 and 6.5 cm long; respectively. The **right and left ovaries** measured 2.6x1.8x1 cm and 2.7x1.5x1.7 cm; respectively with grayish white cut sections. Tumour deposits were seen on the surface of the right tube and right ovary. A portion of **omental tissue** measured 40x14x1.5 cm showing multiple firm tumour nodules; the largest measuring 2.5x1 cm showing grayish white cut section.
2. **Peritoneal nodules**; formed of three nodules; the largest measuring 17x13x4 mm showing firm grayish white cut section.



MICROSCOPIC:

- Serial sections examined from the **panhysterectomy specimen** received revealed a polypoid tumour mass; in the uterine cavity arising from the fundus; spreading along the endometrial lining and formed of a mixture of epithelial and mesenchymal elements. The epithelial elements are formed of irregular glands focally thrown into papillae lined by pleomorphic malignant columnar epithelial cells showing high nucleo/cytoplasmic ratio and hyperchromatic or vesicular nuclei with prominent nucleoli and scattered mitotic figures. The mesenchymal elements are formed of spindle-shaped stromal malignant tumour cells showing hyperchromatic or vesicular nuclei with scattered mitotic figures. Multiple rhabdomyoblasts (with multinucleated forms) were detected together with focal chondroid areas, calcification and bony areas. The tumour mass showed focal areas of tumoural necrosis and hemorrhage.

ميدان التحرير - 2 شارع طلعت حرب - الدور الثالث - ت: 25749601

برج الحرية - الدور الرابع - المعادي - ت: 23586462

23586462 - ت: المعادي - الدور الرابع - برج الحرية

25749601 - ت: شارع طلعت حرب - الدور الثالث - ميدان التحرير

22/6/2026
Jal

Thanks for reference,

DIAGNOSIS: Uterus, tubes and ovaries, Panhysterectomy:
- Polypoid Malignant Mixed Mullerian Tumour (Carcinosarcoma) With Heterologous Stromal Elements, T3, Infiltrating More Than 1/2 Thickness Of The Myometrial Wall And Infiltrating Substantia Propria Of The Upper Cervix. Tumour Deposits Involving Right Tube As Well As Right And Left Ovaries. Free Left Tube From Tumour Deposits. Associated Cervicitis With Retention Cysts, Right Tiny Paratubal Cyst Of The Paramesonephric Type, Left Ovarian Hemangioma.
- Omentum, Omentectomy, Tumour Deposits.
- Pelvic peritoneal nodules, Excision biopsy, Tumour Deposits.

Tumour tissue was seen infiltrating more than half thickness of the underlying myometrial wall without reaching to the perimetrial surface. Tumour tissue was also seen extending to the endometrial lining of the lower uterine segment. Lymphatic tumour emboli were also detected. The cervix showed ectocervical stratified squamous epithelial covering and endocervical columnar lining overlying fibrotic substantia propria mildly infiltrated by chronic inflammatory cells and entangling rounded as well as cystically dilated mucus-secreting glands with retention cysts formation. Tumour tissue was seen infiltrating the substantia propria of upper cervix. The right tube revealed deposits of tumour tissue on its external surface together with a tiny paratubal cyst; formed of a thin fibrotic wall lined by low columnar ciliated epithelial cells. The left tube revealed unremarkable changes. Sections from both ovaries revealed moderately cellular stroma entangling multiple thin and thick walled vessels together with corpora albicantia. The right ovary showed small deposits of tumour tissue. The left ovary showed tumour deposits and a small hemangioma; formed of dilated thin walled small vessels. No evidence of blood vessel invasion by tumour tissue could be detected. Serial sections examined from the omental tissue revealed deposits of tumour tissue formed of irregular glands showing the same features seen in the uterine tumour tissue. Serial sections examined from the peritoneal nodules revealed pieces of fibrofatty tissue showing deposits of tumour tissue; showing the same features seen in the uterine tumour.